

D13 – Office Visit Note

M.K. Osei, MD | Internal Medicine | 17 Mar 2022

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 03/17/2022 | Provider: M.K. Osei, MD

Visit Type: Established Patient / Urgent

Practice: Norfolk Veterans Medical Associates, Norfolk VA

CHIEF COMPLAINT: Intermittent right arm weakness and slurred speech x 2 weeks.

HISTORY OF PRESENT ILLNESS:

65-year-old retired USN Senior Chief Petty Officer with multiple cardiovascular risk factors presenting urgently with transient neurological symptoms. Reports 3 discrete episodes over the past 2 weeks of right arm weakness and slurred speech, each lasting several minutes with full resolution. One episode also included transient unilateral visual blurring. No loss of consciousness. No headache during or after episodes. No fevers. No recent head trauma. No current symptoms at the time of this visit. Self-presented today out of concern.

CURRENT MEDICATIONS:

- Omeprazole 20 mg PO daily
- Metoprolol succinate 25 mg PO daily
- Rivaroxaban 20 mg PO daily
- Atorvastatin 40 mg PO daily
- Aspirin 81 mg PO daily
- Isosorbide mononitrate 30 mg PO daily (added Oct 2020 per cardiology)
- Tamsulosin 0.4 mg PO daily
- Topiramate 25 mg PO daily
- Naproxen 500 mg PO BID PRN
- Diclofenac gel PRN (knees)
- Flunisolide nasal spray PRN
- Lidocaine 5% patch PRN (knees, added Jan 2022)
- CPAP 10 cmH2O nightly

PHYSICAL EXAMINATION:

Vitals: HR 70 | BP 142/86 | RR 16 | SpO2 97% RA | Wt 92 kg

General: Anxious but in NAD at rest. No current symptoms.

HEENT: No facial droop. Symmetric face. Tongue midline.

CV: Rate-controlled, regular. No murmurs. No carotid bruits.

Lungs: Clear bilaterally.

Neuro: Cranial nerves II-XII intact. Strength 5/5 bilaterally with no drift. Sensation intact to light touch and pinprick. Speech fluent and clear. Coordination intact. Gait normal. NIHSS 0 at rest.

IN-CLINIC STUDIES:

CT head w/o contrast (today, Norfolk Radiology): No acute hemorrhage, no large-territory infarct.

ASSESSMENT & PLAN:

1. Transient neurological symptoms - 3 stereotyped episodes over 2 weeks of right arm weakness, slurred speech, and one episode of visual blurring, each lasting minutes with full resolution.

Concerning for transient ischemic attack versus other CNS pathology. CT head reassuring acutely. MRI brain with and without contrast ordered urgently for further evaluation. Urgent neurology referral placed to Dr. B. Okafor, Norfolk Neurology. Patient counseled to STOP DRIVING until cleared by neurology - written instructions provided. Patient is already on dual cardiovascular protection (ASA 81 + rivaroxaban). Continue current regimen.

2. Ischemic heart disease, stable angina - per cardiology (Dr. Mensah): nuclear stress test (Sep 2020) was positive for inducible ischemia. Cardiac catheterization (Oct 2020) demonstrated non-obstructive CAD; medical management. Diagnosis stable angina. Isosorbide mononitrate 30 mg daily added by cardiology Oct 2020. Currently stable.
3. Atrial fibrillation, OSA, GERD, BPH, migraines, PTSD - all stable on current regimens. No changes.

Labs/Orders: MRI brain w/ and w/o contrast (urgent), lipid panel, HbA1c, BMP

Follow-up: Urgent neurology with Dr. Okafor. PCP RTC 4 weeks pending neurology evaluation. Re

Electronically signed by: M.K. Osei, MD

Date: 03/17/2022 14:40

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